



PATIENT'S NAME (Last, First Middle Name)

ADMISSION DATE/TIME

AGUINALDO, Jose Ilustre Ramos

August 15, 2026 1157H

PIN

BIRTHDATE

AGE/GENDER

ROOM/BED

REQUESTING PHYSICIAN

3000137558

July 25, 1968

58y/M

GC-NCCU-RP302-BNCCU02

Laxamana, Lina Cortez, M.D.

VISIT NUMBER 1004168978

Hospital Course, Medications Received During Confinement, DIAGNOSTIC TESTS AND PROCEDURES (Ordered Date), Discharge Information

Rojas, Gabriel Geo Villahermosa (MD) (Signed 20-Aug-2026 09:04)

Authored: Admitting History, Past Medical, Surgical, and Family History, Personal & Social History, and Risk Screen, Review of Systems, Physical Examination On Admission, Admitting & Discharge Diagnosis, Hospital Course, Medications Received During Confinement, DIAGNOSTIC TESTS AND PROCEDURES (Ordered Date), Discharge Information

Last Updated: 20-Aug-2026 09:04 by Rojas, Gabriel Geo Villahermosa (MD)

References:

1. Data Referenced From "Admission Note Nursing" 17-Aug-2026 03:59

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Physician Discharge Summary, Inpatient

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3000137558	July 25, 1968	58y/M	GC-NCCU-RP302-BNCCU02	Laxamana, Lina Cortez, M.D.
VISIT NUMBER 1004168978				

Diagnostic Cardiology:

- 2D Echo W/ Doppler, 15-Aug-2026
- Venous Compression Test, 15-Aug-2026
- Electrocardiography, 16-Aug-2026
- Arterial Duplex Scan, 17-Aug-2026

Others:

- ABG Analysis, 15-Aug-2026
- ABG Analysis, 16-Aug-2026
- ABG Analysis, 16-Aug-2026
- ABG Analysis, 16-Aug-2026
- Tele - Digital EEG Monitoring, 16-Aug-2026

Guide to Access Results:

• TO ACCESS RESULTS:

You may view your results through the eHealth Hub. If you do not have access yet, please create an account online via <https://ehubapp.stlukes.com.ph/> or contact our support team at +63-2-8789-7700 Loc. 6099 or through email at ehhealth@stlukes.com.ph for assistance.

Discharge Information:

DISCHARGE DISPOSITION:

- Discharge Disposition Still admitted
- Discharge Location Still admitted

Get Medication List:

• Medication List

Still admitted

REFERRAL INFORMATION:

- Patient Needs Referral no

DISCHARGE INSTRUCTIONS:

- Discharge Activity Still admitted
- Diet Still admitted
- Additional Discharge Instructions Still admitted

Electronic Signatures:

Laxamana, Lina Cortez (MD) (Signature Pending)

Co-Signer: Admitting History, Past Medical, Surgical, and Family History, Personal & Social History, and Risk Screen, Review of Systems, Physical Examination On Admission, Admitting & Discharge Diagnosis,

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VISIT NUMBER 1004168978

- Renal Panel 2, 16-Aug-2026
- Vitamin D Total, 16-Aug-2026
- CPK-Total, 16-Aug-2026
- Creatinine Kinase - MM, 16-Aug-2026
- Sodium and Potassium, 16-Aug-2026
- Sodium and Potassium, 16-Aug-2026
- MTB - PCR (Mycobacterium tuberculosis), 17-Aug-2026
- Creatinine, 17-Aug-2026
- Gram Stain, 17-Aug-2026
- Magnesium Level, 17-Aug-2026
- Phosphorus Level, 17-Aug-2026
- Serum Osmolality, 17-Aug-2026
- Sodium and Potassium, 17-Aug-2026
- Urinalysis, 17-Aug-2026
- Sodium and Potassium, 17-Aug-2026
- Sodium and Potassium, 17-Aug-2026
- Renal Panel 2, 18-Aug-2026
- Complete Blood Count, 18-Aug-2026
- Sodium and Potassium, 18-Aug-2026
- Gram Stain, 18-Aug-2026
- Sodium and Potassium, 18-Aug-2026
- Calcium, Ionized, 19-Aug-2026
- Creatinine, 19-Aug-2026
- Magnesium Level, 19-Aug-2026
- Phosphorus Level, 19-Aug-2026
- Sodium and Potassium, 19-Aug-2026
- Sodium and Potassium, 19-Aug-2026
- Complete Blood Count, 20-Aug-2026
- Calcium, Ionized, 20-Aug-2026
- Creatinine, 20-Aug-2026
- Sodium and Potassium, 20-Aug-2026

Diagnostic Imaging:

- XR Chest PA, 15-Aug-2026
- CT Brain-Plain, 15-Aug-2026
- XR Chest PA, 15-Aug-2026
- XR Chest PA, 15-Aug-2026
- XR Chest PA, 16-Aug-2026
- US Whole Abdomen Prostate, 17-Aug-2026
- CT Brain-Plain, 18-Aug-2026
- XR Chest PA, 19-Aug-2026
- CT Brain-Plain, 19-Aug-2026
- CT Brain-Plain, 20-Aug-2026

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lactulose 3.3g/5mL 120mL Syrup (DUPHALAC) 30 milliLiter(s) NasoGastric Tube at bedtime
 metoclopramide Injectable 5 milliGRAM(s) IntraVenous every 8 hours
 ipratropium bromide + salbutamol sulfate 500mcg/2.5mg/2.5mL Inhalation (DUAVENT) 3.25 milliLiter(s) Inhalation every 8 hours
 niCARDipine HCl Infusion 1 mg/hr IntraVenous Continuous <Continuous>
 diazePAM Injectable 5 milliGRAM(s) IntraVenous every 6 hours
 vitamin B complex Injectable (SANNOVIT) 3 milliLiter(s) IntraVenous daily
 fentaNYL citrate Infusion (SUBLIMAZE) 30 mcg/hr IntraVenous Continuous <Continuous>
 fentaNYL citrate Injectable (SUBLIMAZE.) 12.5 MICROGram(s) IntraVenous every 6 hours
 dexmedetomidine premix 0.4 mcg/kg/hr IntraVenous Continuous <Continuous>
 levETIRAcetam IVPB (KEPPRA.) 500 milliGRAM(s) IntraVenous Piggy Back every 12 hours
 niCARDipine HCl Infusion 1 mg/hr IntraVenous Continuous <Continuous>
 pantoprazole Injectable (PANTOLOC IV) 40 milliGRAM(s) IntraVenous daily
 piperacillin sodium+tazobactam sodium IVPB (PIPTAZ) 4.5 Gram(s) IntraVenous Piggy Back every 6 hours
 paracetamol IVPB 1 Gram(s) IntraVenous Piggy Back every 6 hours
 mannitol 20% Injectable 155 Gram(s) IntraVenous Push ONCE
 mannitol 20% Injectable 320 milliLiter(s) IntraVenous Push every 4 hours
 mannitol 20% Injectable 165 milliLiter(s) IntraVenous Push every 4 hours
 pantoprazole Injectable (PANTOLOC IV) 40 milliGRAM(s) IntraVenous ONCE
 sodium chloride 0.9% Infusion 1000 milliLiter(s) IntraVenous Continuous <Continuous>
 niCARDipine HCl Infusion 3 mg/hr IntraVenous Continuous <Continuous>
 niCARDipine HCl Infusion 2 mg/hr IntraVenous Continuous <Continuous>
 sodium chloride 0.9% Infusion 1000 milliLiter(s) IntraVenous Continuous <Continuous>

DIAGNOSTIC TESTS AND PROCEDURES (Ordered Date):

Laboratory Test:

- Blood Typing, 15-Aug-2026
- PRBC, Filtered/Leukoreduced, 15-Aug-2026
- Antibody Screen, 15-Aug-2026
- Blood Cross Match, 15-Aug-2026
- Blood Cross Match, 15-Aug-2026
- Complete Blood Count, 15-Aug-2026
- Calcium, Ionized, 15-Aug-2026
- Magnesium Level, 15-Aug-2026
- Renal Panel 2, 15-Aug-2026
- Glycohemoglobin (HPLC), 15-Aug-2026
- Uric Acid, 15-Aug-2026
- Partial Thromboplastin Time (PTT), 15-Aug-2026
- Prothrombin Time (PT), 15-Aug-2026
- Culture and Sensitivity (Others), 15-Aug-2026
- Sodium and Potassium, 15-Aug-2026
- Urinalysis, 15-Aug-2026
- Complete Blood Count, 16-Aug-2026
- Phosphorus Level, 16-Aug-2026

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NON-CLINICAL
PHYSICIAN

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Laxamana, Lina Cortez, M.D.

VISIT NUMBER 1004168978

lactulose 3.3g/5mL 120mL Syrup (DUPHALAC) 30 milliLiter(s) NasoGastric Tube 2 times per day
amLODIPine Besylate (NORVASC) 10 milliGRAM(s) NasoGastric Tube <User Schedule>
acetylCYSTEINE Inhalation (FLUIMUCIL Inh) 3 milliLiter(s) Inhalation 2 times per day
lamoTRigine (LaMICTal) 50 milliGRAM(s) NasoGastric Tube daily
metoprolol TARTRATE 25 milliGRAM(s) NasoGastric Tube <User Schedule>
dexlansoprazole Delayed Release (DEXILANT) 30 milliGRAM(s) NasoGastric Tube <User Schedule>
Vitamin B1+ B6+ B12 250 mg-250 mg-1 mg (POLYNERV 1000) 1 Tablet(s) NasoGastric Tube daily
rosuvastatin (CRESTOR) 40 milliGRAM(s) NasoGastric Tube daily
acetylCYSTEINE Effervescent Granules (EXFLEM Granules.) 600 milliGRAM(s) NasoGastric Tube 2 times per day
piperacillin sodium+tazobactam sodium IVPB (PIPTAZ) 4.5 Gram(s) IntraVenous Piggy Back every 6 hours
paracetamol IVPB 1 Gram(s) IntraVenous Piggy Back every 6 hours
ipratropium bromide 500mcg/2mL Inhalation (AtroVENT UDV) 4 milliLiter(s) Inhalation every 8 hours
amino acids + glucose + electrolytes + vitamin B1 Infusion (BFLUID) 1000 milliLiter(s) IntraVenous Continuous <Continuous>
diazepam Injectable 5 milliGRAM(s) IntraVenous every 6 hours
levETIRacetam IVPB (KEPPRA.) 500 milliGRAM(s) IntraVenous Piggy Back every 12 hours
niCARDipine HCl Infusion 2.5 mg/hr IntraVenous Continuous <Continuous>
fentaNYL citrate Infusion (SUBLIMAZE) 12.5 mcg/hr IntraVenous Continuous <Continuous>
febuxostat (ATENURIX) 80 milliGRAM(s) NasoGastric Tube daily
fentaNYL citrate Infusion (SUBLIMAZE) 30 mcg/hr IntraVenous Continuous <Continuous>
propofol Injectable (FRESOFOL MCT/LCT 1%.) 20 milliGRAM(s) IntraVenous <See Task>
cholecalciferol Capsule (LAC cholecalciferol) 2000 International Unit(s) NasoGastric Tube daily
Hypoglycemic Protocol (Dextrose 50% in water) (D50W EURO.) null IntraVenous <See Task>
sodium chloride 2% Infusion 1000 milliLiter(s) IntraVenous Peripheral Line <Continuous>
mannitol 20% Injectable 150 milliLiter(s) IntraVenous Push ONCE
metoclopramide Injectable 5 milliGRAM(s) IntraVenous every 8 hours
potassium chloride (7.5%) 5mEq/5mL 120mL Syrup (POTASSIUM CHLORIDE) 20 milliLiter(s) NasoGastric Tube 3 times per day
paracetamol IVPB 1 Gram(s) IntraVenous Piggy Back ONCE
potassium phosphate Infusion 4 mEq/hr IntraVenous Continuous <Continuous>
niCARDipine HCl Injectable 1 milliGRAM(s) IntraVenous Push ONCE
paracetamol IVPB 1 Gram(s) IntraVenous Piggy Back every 6 hours
ceFAZolin IVPB 2 Gram(s) IntraVenous Piggy Back ONCE
fentaNYL citrate Injectable (SUBLIMAZE.) 100 MICROGram(s) IntraVenous ONCE
rocuronium Injectable 50 milliGRAM(s) IntraVenous ONCE
propofol Injectable (FRESOFOL MCT/LCT 1%.) 50 milliGRAM(s) IntraVenous ONCE
piperacillin sodium+tazobactam sodium IVPB 4.5 Gram(s) IntraVenous Piggy Back ONCE
mannitol 20% Injectable 165 milliLiter(s) IntraVenous Push ONCE
paracetamol IVPB 1 Gram(s) IntraVenous Piggy Back ONCE
metoclopramide Injectable 5 milliGRAM(s) IntraVenous every 8 hours
bromocriptine mesylate 2.5 milliGRAM(s) NasoGastric Tube daily
amLODIPine Besylate (NORVASC) 5 milliGRAM(s) NasoGastric Tube <User Schedule>
bromocriptine mesylate 2.5 milliGRAM(s) NasoGastric Tube daily

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NPi 1.8), prompting the administration of Mannitol 150ml IV and resumption of fentanyl for a STAT cranial CT. The scan revealed stable right thalamic and midbrain hemorrhage with a mild progression in midline shift to 10.4mm, but no frank uncal herniation. During transfer from the CT, the internal jugular catheter was accidentally dislodged; bleeding was promptly controlled with direct pressure, and anesthesia was referred for a new IV insertion. The patient subsequently experienced an episode of paroxysmal sympathetic hyperactivity featuring hypertension and tachycardia, which resolved rapidly after administering Propofol 20mcg IV and reconnecting the nicardipine drip. On reassessment, the anisocoria and left pupillary reactivity significantly improved (L 3.53mm, NPi 3.1), while the motor examination remained stable with right-sided localization and left-sided abnormal extension to pain. 20-Aug-2026

- Date
- Hospital Course

The patient is currently Day 5 post-ictus and Day 5 post-operation. Following a CT scan, it was noted that the internal jugular (IJ) catheter was removed yesterday. During the course, the patient experienced episodes of chills, hypertension, and tachycardia, which were considered to be manifestations of Paroxysmal Sympathetic Hyperactivity (PSH). A Propofol push was administered, and there was no recurrence of the episodes afterward.

The patient's neurologic status remained stable. The patient is maintained at a RASS of -2 on a Fentanyl drip at 15 mcg/hr. Neurologic assessment shows a GCS of 8 (E1VTM4). There is no eye opening to pain. Pupils are isocoric at 3 mm and briskly reactive to light bilaterally. On motor examination, the right extremities exhibit occasional spontaneous movement, while the left upper extremity abnormally extends and the left lower extremity withdraws.

Medications Received During Confinement:

Medications Received during Confinement:

- Medication Summary (Clinical Summary)

metoclopramide Injectable 10 milliGRAM(s) IntraVenous every 8 hours
bromocriptine mesylate 2.5 milliGRAM(s) NasoGastric Tube <User Schedule>

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- Date
- Hospital Course

oxygen saturation at 100%. The external ventricular drain (EVD) was maintained at head level, draining light sanguineous cerebrospinal fluid.
17-Aug-2026

At 6:00 AM, the patient was given Propofol 25 mg IV due to tachycardia and was maintained on a Fentanyl drip at 10 mcg/hr. At 7:00 AM, the Nicardipine drip was resumed due to episodes of elevated blood pressure, reaching as high as 5 mg/hr.

On current examination while maintained on the Fentanyl drip at 10 mcg/hr, the patient has a RASS of -4. The patient exhibits unsustained eye opening to pain and does not follow commands. Pupils measure 2.67 mm (NPI 4.2) on the right and 2.88 mm (NPI 3.4) on the left, with an inconsistent visual threat response. Extraocular movements show slightly hypotropic and roving eyes. Motor assessment reveals the left upper extremity extending to pain and the left lower extremity withdrawing to pain. Right-sided motor strength is 3+/5 in the upper extremity and 3-/5 in the lower extremity. Bilateral Babinski reflex is present.

- Date
- Hospital Course

18-Aug-2026
patient was noted to have a more depressed sensorium, accompanied by an episode of hiccups, tremors on the right upper extremity, and being warm to touch. Due to these findings, an early dose of Diazepam 5 mg IV was administered.

On neurologic examination, there was no eye opening to pain, no regard, and no visual threat, though the patient inconsistently followed commands (e.g., squeezing the right hand). Pupils measured 2.32 mm (NPI 4) on the right and 2.24 mm (NPI 4) on the left, and were generally midline with an occasional preferential gaze to the right. Gag reflex and spontaneous breathing were intact. Motor assessment revealed 3/5 strength in the right upper and lower extremities. The left upper extremity showed abnormal extension to pain, while the left lower extremity withdrew to pain. Bilateral Babinski reflex was present.

- Date
- Hospital Course

19-Aug-2026
After discontinuing fentanyl, the patient developed anisocoria with a poorly reactive left pupil (L 5.17mm).

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- Admission Weight (lbs) 142 lb
- Weight Method measured
- BMI (kg/m2) 16.4
- BSA (m2) 1.94 M2

PERTINENT FINDINGS PER SYSTEM:

- General Survey Altered Sensorium
- HEENT essentially normal
- Chest/Lungs essentially normal
- CVS essentially normal
- Abdomen essentially normal
- GU (IE) essentially normal
- Skin/Extremities essentially normal
- Neuro Exam National Institute of Health Stroke scale 26

ADMITTING & DISCHARGE DIAGNOSIS:

Admitting Dx:

Thalamic hemorrhage (I61.0): Onset Date: 15-Aug-2026, Status: Active, Present on admission verified (POA), ICD-10: I61.0

Intracerebral hemorrhage (I61.9): Onset Date: 15-Aug-2026, Status: Active, Present on admission verified (POA), ICD-10: I61.9

Discharge Dx:

Intracerebral hemorrhage (I61.9): Onset Date: 20-Aug-2026, Status: Active, ICD-10: I61.9

Hospital Course:

- Date 15-Aug-2026
- Hospital Course Patient seen and examined. Assessed and referred to neurosurgery for emergency procedure. Patient tolerated the procedure and was transferred to Neurocritical Care Unit post op

- Date 16-Aug-2026
- Hospital Course The patient had no untoward events, febrile episodes, or recurrence of restlessness. The patient was maintained at a RASS of 0 on a Fentanyl drip at 15 mcg/hr. Neurologic assessment showed a GCS of 8 (E1VTM6). There was no spontaneous eye opening, but the patient was able to follow complex commands (consider lid apraxia); on passive eye opening, extraocular movements were full. Motor examination revealed 3+/5 strength in the right extremities, while the left extremities withdrew to pain. Blood pressure was controlled at 135/82 mmHg on a Nicardipine drip at 5 mg/hr. Other vital signs were stable with a heart rate of 91 bpm, respiratory rate of 22 cpm, and

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- No disease found (Z71.1): Status: Active

PAST SURGICAL HISTORY:

Past Surg Hx:

- No disease found (Z71.1): Status: Active

FAMILY HISTORY:

- No disease found (Z71.1): Status: Active

CIGARETTE USE:

- Cigarette Smoking Status unknown if ever smoked⁽¹⁾

CAFFEINE USE:

- Caffeine Use Status does not use caffeine⁽¹⁾

ALCOHOL USE:

- Alcohol Use Status alcohol never used⁽¹⁾

RECREATIONAL DRUG USE:

- Recreational Drug/ Medication Use Status recreational drug/medication never used⁽¹⁾

Review of Systems:

PERTINENT FINDINGS ON ADMISSION:

- Pertinent Signs and Symptoms on Admission Left sided weakness

Physical Examination On Admission:

VITAL SIGNS:

- Temp (degrees F) 98.1 degrees F
- Temp (degrees C) 36.7 degrees C
- Temperature Site tympanic
- Heart Rate (beats/min) ↑ 114 bpm
- Respiration (breaths/min) 16 breaths/min
- SpO2 (%) 100 %
- Patient On room air
- Systolic (mm Hg) ↑ 166 mm Hg
- Diastolic (mm Hg) ↑ 111 mm Hg
- Mean (mm Hg) ↑ 129 mm Hg
- Height (cm) 197.6 cm
- Height Method measured
- Admission Weight (kg) 64.4 kg

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CHIEF COMPLAINT:

• Chief Complaint

Left sided weakness

HISTORY OF PRESENT ILLNESS:

Significant Admitting History: Patient is a 58 year old male, right handed, MRS 1, known with history of stroke, HTN, coming in from Taguig City General Hospital as per family's choice for left sided weakness

8/15 0100H patient was awake watching a movie with wife where he was seen with no headache, dizziness, vomiting, imbalance, doubling of vision, aspiration episodes, unilateral numbness. There were no noted facial asymmetry or dysarthria eventually went to sleep at 0130H.

8/15 0300H, patient was about to stand up to go to the restroom however noted difficulty standing due to generalized weakness however noted by wife to be leaning to the left associated with left facial asymmetry, slurring of speech, and retching episodes with no headache, vomiting, loss of consciousness or changes in sensorium. No vitals were taken. Ambulance was immediately called.

0330H, patient was picked up by ambulance unrecalled BP then noted increase sleeping time, incomprehensible sound, progression of weakness on left arm and leg. No vomiting or headache.

0345H, arrived Taguig City General Hospital with SBP 230

CT brain plain immediately done ~ 48 minutes post ictus (0345H) (uploaded in vuepacs ID 42630)

Hyperdensity on right thalamocapsular region with IVH extension on lateral ventricle (anterior and posterior horn) and left temporal horn with perilesional edema

Patient was hooked to nicardipine drip

Patient was given mannitol 150ml as loading dose 0415H then given 300ml at 0900H

BP trends while at Taguig - fluctuating 150-185

Noted 4 episodes of vomiting - previously ingested food (blood tinged on last vomitus)

Patient was advised ICU admission for neurosurgical evaluation then decided transfer to our institution.

• Referred from another health care institution (HCI): No

ALLERGY, INTOLERANCE, ADVERSE EVENT:

Allergies:

• No Known Allergies:

PAST MEDICAL HISTORY:

Past Medical Hx:

This report remains provisional and is pending electronic validation by the Attending Physician.

THE DOCUMENT SERVES AS THE PATIENT'S CLINICAL RESUME AND CLINICAL ABSTRACT.

For concerns, please contact the Emergency Care Services at (02) 8789-7700 ext. 1032 / 1035.

ST. LUKE'S MEDICAL CENTER
GLOBAL CITY
MEDICAL RECORDS MANAGEMENT SERVICES
CERTIFIED TRUE COPY

AUG 16 2026

DARWIN F. GONZALES
NON-CLINICAL
SUPPORT ASSOCIATE I

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